

Public Health Emergency Operations Center
« COUSP RDC »

MVE-17 Incident Management System

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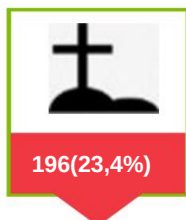
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°32/MVB_15/06/2026

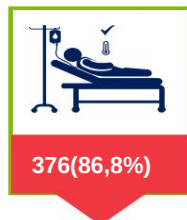
Affected provinces (3)	EAST, NORTHWEST & SOUTHWEST
Affected Health Zones (31)	- Ituri (20/36) : Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Commander, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia. - North Kivu (10/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi and Mabalako. - Sud-Kivu (1/34) : Miti-Murhesa
Date the report	June 15, 2026
Publication date	June 16, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



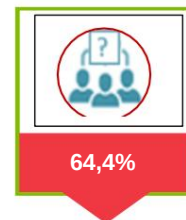
Patients in
isolation -
hospitalisation



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

*Data is being harmonized to include confirmed cases still in the community as well as the cases that escaped inclusion in the overall count.

Suspicious deaths occurring in the community are subject to investigation. further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- Twenty-nine (29) new confirmed cases including 4 deaths, were reported in the province of Ituri during the day of June 15, 2026. These confirmed cases are distributed in the **ZS Mongbwalu (10), Rwampara (7), Nizi (5), Nyankunde (3), Lita (3), Komanda (1).**
- One (1) patient** (ZS Mongbwalu) with Ebola virus disease has been declared cured after obtaining negative control tests.
- Attack on EDS teams in AS PLUTO** (ZS Mongbwalu). The EDS was carried out after the intervention law enforcement.
- 5 PoE/PoC providers (3 PNHF, 1 ICCN and the Driver) were sequestered by uniformed elements; the latter accuse them of spreading the disease as actors in the response.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements toward Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a population

total estimated at nearly 15 million inhabitants, with massive internal displacements and cross-border flows to neighboring countries.

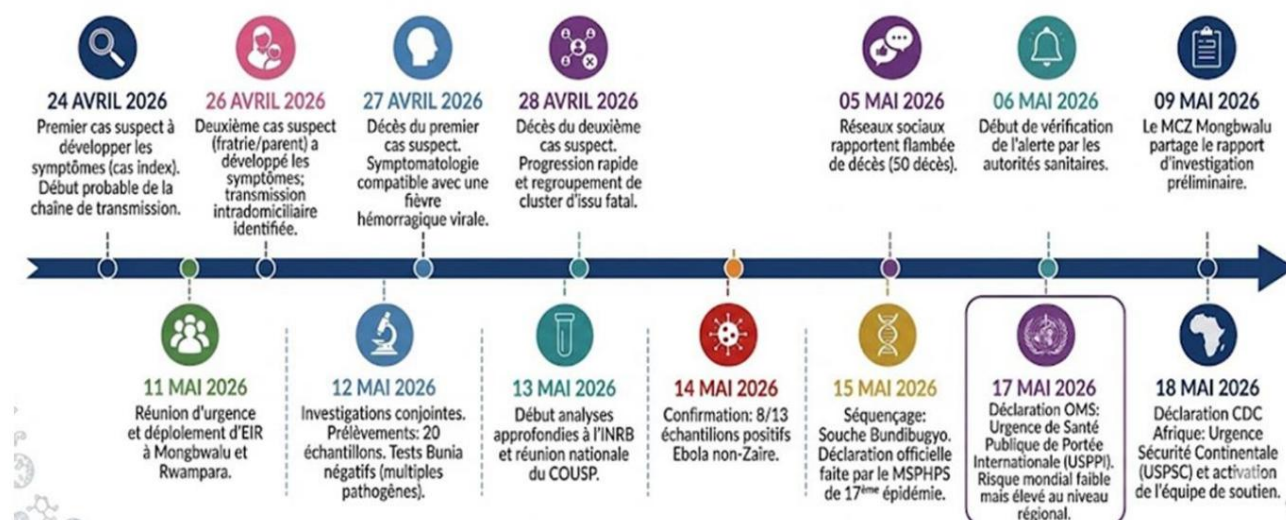


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 15, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of June 15, 2026.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New cas confirmed (24h)
Ituri	767	157	20,5%	20 out of 36 (55.5%)	29
North Kivu	67	38	56,7%	10 out of 34 (29.4%)	0
South Kivu	3	1	33,3%	1 out of 34 (2.9%)	0
Total	837	196	23,4%	31 out of 104 (29.8%)	29

- At the end of the day on June 15, 2026, no new health zones had been affected. The number of affected health zones remained at 31/104 (section 3.1). Approximately 91.6% of the confirmed cases reported in the three affected provinces originated in Ituri province, making it the epicenter. Furthermore, North Kivu province remained the most affected in terms of case fatality rate (56.7%), while Ituri province...

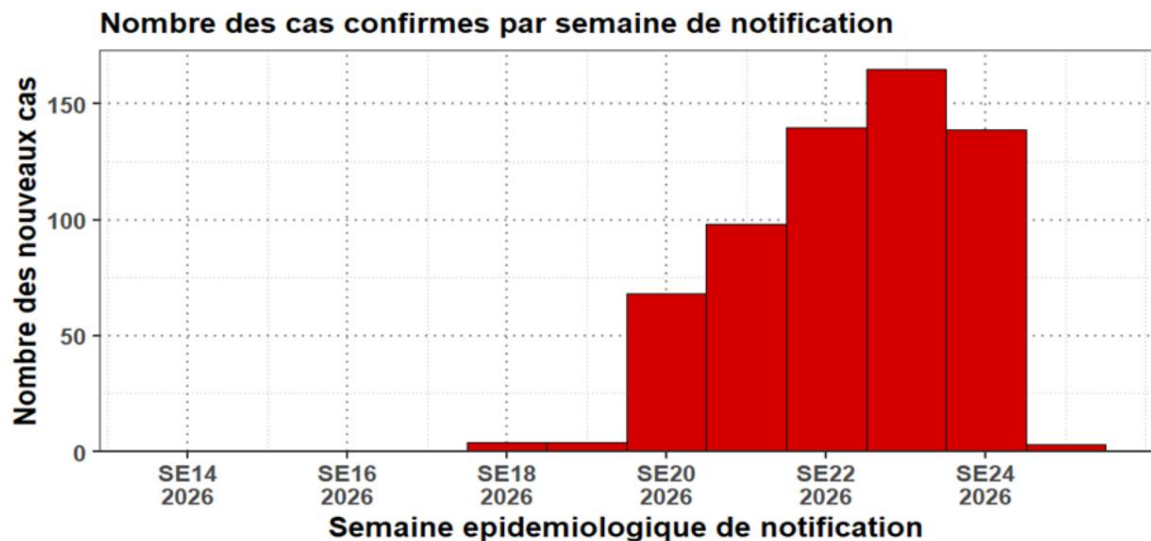
shows a low case fatality rate (20.5%) despite the significant number of deaths recorded. Furthermore, the province of South Kivu has not reported any confirmed cases since May 26, 2026 (Table 1).

3.2 Distribution by health zone (Cumulative as of 15/06/2026)

- Ituri Province (767 cases):** Bunia (215), Rwampara (159), Mongbwalu (185), Nyankunde (45), Nizi (16), Bambu (7), Komanda (7), Lita (9), Mangala (5), Kilo (4). (2), Gety (1), Kambala (1) and Nia-Nia (1 case). (Ninety-four additional cases are undergoing ventilation.

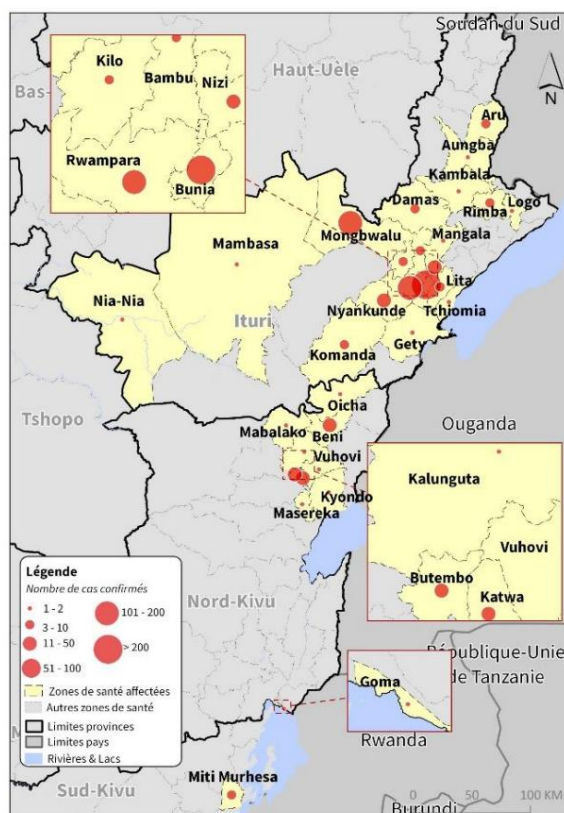
- **North Kivu Province (67 cases):** Katwa (25), Butembo (19), Beni (14), Oicha (2), Kyondo (2), Kalunguta (1), Goma (1), Maserka (1), Vuhovi (1) et Mabalako (1).
- **Province of South Kivu (3 cases) :** Miti-Murhesa (3).

3.3 Time analysis: We observe an increasing number of confirmed cases from one week to the next, reflecting continued transmission of the disease in the community. A rapid geographical expansion of the epidemic is feared if public health measures are not implemented quickly.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 621.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 15, 2022

Table 2. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province as of June 15, 2026.

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (%)
ITURI			
Bunia	215	20	9,3%
Rwampara	159	27	17,0%
Mongbwalu	185	83	44,9%
Legumes	45	4	8,9%
Bamboo	7	2	28,6%
Understand	3	1	33,3%
Kilo	4	1	25,0%
Nizi	16	1	6,3%
Mangala	5	3	60,0%
Ladies	4	0	0,0%
Aungba	2	1	50,0%
Tide		0	0,0%
Team	17	1	14,3%
Liter	9	0	0,0%
Logo	2	0	0,0%
Washing	2		50,0%
Jungle	3	1	0,0%
Tchomia	2	0	0,0%
Flounder			100,0%
Nanny			100,0%
Other ZS (undisclosed data)	1194	01110	10,6%
Ituri subtotal	767	157	20,5%
NORTH KIVU			
Cut	25	16	64,0%
Me	14	10	71,4%
Butembo	19	7	36,8%
Oicha	2	2	100,0%
Kalunguta	1	1	100,0%
Kyondo	2	1	50,0%
Ten	1	0	0,0%
Masereka	1	0	0,0%
Vuh	1		100,0%
Be anxious.	1	10	0,0%
North Kivu subtotal	67	38	56,7%
SOUTH KIVU			
Miti-Murhesa	3	1	33,3%
South Kivu subtotal	3	1	33,3%
TOTAL	837	196	23,4%

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri :

- Visit of the Director General of the National Institute of Public Health (INSP) to the Elikya treatment center for the evaluation of the implementation of care activities.
- Holding of the coordination meeting on monitoring the activities carried out in the field by the partners technical and financial aspects across the pillars of the response.
- Two brainstorming sessions on increasing reception capacities in the different CTEs and the dynamics of the logistics of the response to the MVE.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 15, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		276	483	7	766
Alerts investigated		188	483	7	678
Investigation rate		68,1%	100.0%	100,0%	88,5%
Alerts confirmed	Alive	131	36	7	174
	Deceased	45	16	0	61
Total suspected cases for today		176	52	7	235

For the day of June 15, 2026, **766 alerts** were reported in the three affected provinces, **678 (88.5%)**

Of these alerts investigated, **235 suspected cases (61 deaths)** were identified.

Table 4. Contact tracing of confirmed cases as of June 15, 2026

Province	Contacts under follow-up (n)	Contacts vus (n)	Follow-up rate (%)
Ituri	5 208	3 397	65,2%
North Kivu	1 187	700	59,0%
South Kivu	80	74	92,5%
Total	6 475	4 171	64,4%

- Continued in-depth investigations and exhaustive listing and monitoring of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and monitored per case).
- 466 new contacts were listed in 3 health zones in Ituri province, including 435 in Rwampara, 21 in Nyankunde and 10 to Tchomia.

Update on checkpoint and point of entry (PoC/PoE) activities

- A total of seven (7) alerts were notified and validated to the PoCs and then investigated.

Two (2) lifeless bodies were intercepted at the Dele and Nyori PoCs and then transported to the cemetery without Swab and without EDS due to the late arrival of the EDS team.

We also note that 5 PoE/PoC providers (3 PNHF, 1 ICCN and the Driver) were held captive by uniformed personnel; the latter accuse them of spreading the disease as actors in the response.

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) in Ituri province, as of June 15, 2026

Indicators	Results
Proportion of activated PoCs	80,0% (44/55)
Number of travelers who passed	56 294
through, % of travelers screened,	95,0%
% of travelers who washed their hands, % of	94,8%
travelers made aware of the risks	97,0%
Number of alerts notified	7
Percentage of alerts investigated or bodies swabbed within 24 hours	0,0%
Number of validated alerts	7
% of results available in 24 hours	0%
Number of alerts transferred to the CT/CTE or for EDS	5
Number of contacts intercepted today	0

4.3 Laboratory

- **Ituri:** 130 new samples collected and analyzed (positivity 22.3%; n=29).
- **South Kivu :** 7 new samples collected and analyzed, all came back negative.

4.4 Infection Prevention and Control (IPC)

- **Ituri:** briefing of 45 service providers in the different health zones of Bunia on general concepts of infection prevention and control (IPC); waste management and general information on Ebola virus disease (EVD); briefing of 42 service providers from the health zones of Mambasa, Rwampara, Komanda; decontamination of 8 health establishments from the health zones of Rwampara, Komanda, Bunia and Mongbwalu.
- **North Kivu :** 38 healthcare facilities (HCFs) supported in the health zones of Butembo, Katwa, Beni and Goma; provision of IPC supplies to Matanda General Referral Hospital; decontamination of 1/2 HCFs (delivery room and postpartum room of Matanda General Referral Hospital); assessment of the degree of exposure of 6/13 providers at Saint Joseph Hospital and Kitatumba General Referral Hospital, including 3 classified as high risk; 97 providers briefed on IPC topics in the health zones of Katwa, Beni and Butembo; decontamination of Kitatumba General Referral Hospital and Saint Joseph Hospital (gap of 1/6 HCFs remain to be decontaminated); IPC scorecard assessment of 7 HCFs in the Beni health zone (Average of 47.4%); closure of IPC training for 11 health providers at the refugee transit site with the support of WHO and UNHCR.
- **South Kivu:** Evaluation score card of 5 ESSs, including 4 in the Katana ZS and 1 in the Mitimurhesa ZS.

4.5 Holistic care

- One (1) patient (Mongbwalu) with Ebola virus disease has been declared cured after obtaining negative follow-up tests.
- Nutritional monitoring of patients in all CTEs.

- Monitoring and nutritional care of patients in all CTEs: 590 hot meals were offered (107 suspected, 119 confirmed, 340 PPL and 24 accompanying persons).

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of June 15, 2026

Indicator	Ituri	North Kivu	South Kivu	Ensemble
Patients in bed (Day -1)	312	44	6	362
Total admissions (24h)	47	7	7	61
Sorties (24h)				
Deceased (Suspects/Confessions)	14	0	0	14
Non-case	17	6	3	26
Healed	1	0	0	1
Escapees (Suspects/Confessions)	6	0	0	6
Transferred to the HGR	0	0	0	0
Total sorties	38	6	3	47
Patients in isolation (End of Day)	321	45	10	376
including confirmed (NC+AC) and	152	15	1	168
suspected cases	169	30	9	208
Overall occupancy rate	97,3%	63,4%	31,3%	86,8%

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of June 15, 2026

KEY INDICATORS	ITURI	SOUTH KIVU	NORTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	2(10%)	1(100%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	53(40,2%)	0	2(100%)
New suspected cases that benefited from SMSPS interventions	20(42,6%)	3(100%)	2(100%)
Suspected cases under follow-up that have received interventions SMSPS	54 (43,5%)	5 (100%)	4(100%)
Laboratory results have been announced	13	2	0
Including positive ones	3	1	0
Number of children separated in daycare who benefited from interventions SMSPS	1 (100%)	0	1(100%)
Number of children separated in the community who benefited from SMSPS interventions	0	0	13
Number of accompanying persons at the CTE who benefited from interventions SMSPS	36	7	0
Number of PPLs who benefited from SMSPS interventions	9	14	12
Number of household and community members who benefited from SMSPS interventions	101	12	0
Number of EDS supported	3	0	0

4.6 Continuity of services.

- Support for strengthening transfusion safety (equipment and materials from CNTS to Bunia) (by WHO).
- Monitoring of the deployment of medicines provided by UG-PDSS and MEMISA Belgium.

4.7 Risk communication and community engagement

North Kivu: 32 community leaders briefed on Risk Communication and Community Engagement (RCCE) activities; 20,704 people reached through visits to 10,252 households, thanks to the mobilization and supervision of 1,305 community actors. These door-to-door activities also resulted in the collection of 51 community alerts in the health zones of Goma, Karisimbi, and Nyiragongo;

9,719 people were reached through mass awareness sessions and educational talks in the health zones of Goma, Karisimbi, and Nyiragongo; 18 radio stations participated, producing 3 formats and broadcasting 2. These activities resulted in the collection of 2 community feedback sessions, all of which were fully clarified (100%).

South Kivu: Support for surveillance during sampling of 6 suspected cases; 32 people sensitized on the importance of sampling and prevention measures for Ebola virus disease; advocacy meeting with 75 (43 men and 32 women) community leaders and village chiefs on their involvement in the fight against Ebola virus disease.

Ituri: Advocacy with the Vice-President of the Provincial Assembly and President of UNADI for their effective involvement in CREC activities in support of the fight against Ebola; the new military Governor of the Ituri province invites the inhabitants to fully adhere to the various barrier measures against Ebola and other measures (adherence to temperature checks, proper hand washing, EDS and isolation at the Ebola Treatment Center if necessary); awareness-raising of the masses of 2,168 (876 men and 1,292 women) on preventive measures against Ebola in different work and living environments in the health zones of Nyankunde, Aru, Bunia; Home visits to 8,080 households by 832 Recos with messages on Ebola prevention measures and reporting of alerts (deaths and living) reaching 8,121 men and 11,626 women in the health zones of Bunia, Nyankunde and Rwampara; 12 Recos conducted educational talks in 16 households and reached 278 people (110 men and 168 women); briefing of 31 people on community-based surveillance for reporting alerts and Ebola prevention measures, use of protective PPE, conduct during disinfection (health zones of Mongbwalu and Nyankunde).

Infodemic Management/Feedback.

Support for other pillars : Surveillance : Support for investigations and contact tracing (Bunia Health Zone, Bigo Health Area). Support for the investigation of a case in the community (Nyakasanza Health Area/Bunia Health Zone). Result: Case investigated, transported to the Ebola Treatment Center, and a preliminary contact tracing list completed. Infection Prevention and **Control (IPC) :** Preparation of a family for participation in household decontamination (Mongbwalu Health Zone, Shun 2 Health Area). Result: Decontamination completed **Management :** To shed light on a suspected case's reluctance to go to the CTE (ZS Bunia, AS Bigo). Result: He promised to go to the CTE.

4.8 Logistics

- Continued monitoring of the development work in the CTE HGR Nyankunde.
- Construction work continues on a new CTE/HGR Bunia building (12 tents are already functional while awaiting the completion of the work).

4.9. Security

- Incident in the Mongbwalu Special Zone (resistance to the EDS): EDS teams were attacked in the Pluto Special Zone. Fortunately, no loss of life was reported.
- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- **Ituri** : Briefing on PSEA for 130 health actors including 87 women, 123 signed the code of ethics and good conduct; PSEA training for UNHAS staff, 27 participants including 10 women.
- **North Kivu**: 37 hygiene workers, including 19 women and 18 men, were sensitized on PSEA and AAP in the Karisimbi health zone.
- **South Kivu**: PSEA briefing of 45 hygiene workers and service providers (including 15 women) in the ZS of Mitimurhesa, and all signed the code of conduct; 98 people (the ACs and members of local, national and international organizations) involved in the CREC were briefed on the PSEA, reporting mechanism and key awareness messages.

5. MAIN CHALLENGES

Challenge	Impact	Action required
No. 1 Reluctance to perform post-mortem sampling (swabs)	Confirmation delay; underestimation of cases	Intensive community engagement; involvement of religious leaders
2 Insufficient capacity in standardized CTEs despite the opening of Rwampara	Potential saturation of existing CTEs	Acceleration of construction of CTE planned
3 Weakness in contact tracing (overall rate 64.4%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4 Insufficient supply of Essential Generic Medicines (EGMs) in targeted General Respiratory Hospitals	Weakening of overall care	A plea for MEG funding
5 Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6 Low increase in alerts in the provinces of Ituri and South Kivu	Delay in detection of cas	Strengthening community surveillance
7 Insufficient financial resources for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action	Responsible pillar	Due date
1 Strengthen awareness-raising and community engagement activities of local leaders	CREC	Continue
2 Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Surveillance/CREC D+3	
3 Briefing of community agents on Ebola surveillance and the SBC (Ituri province)	Surveillance/CREC D+3	
4 Supervised support for national and provincial coordination in the technical pillars	Coordination	Continue
5 Catch-up in contact tracing in Ituri remains weak.	Surveillance	J+7
6 Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	J+2
7 Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	J+3
8 Plan for catching up on samples awaiting analysis in North Kivu Laboratory		J+2

**FOR ALL
INFORMATION
ADDITIONAL,
PLEASE CONTACT**

For the National Institute of Public Health (INSP) of the DRC

The Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: dieudonne.mwamba@insp.cd

Tel. : +243 816 040 145

The COU-SP Coordinator

Prof. NGANDU Christian

Email: Christian.ngandu@insp.cd

Tel.: +243998091915

L'Incident Manager

Prof. Pierre AKILIMALI

Email: pierre.akilimali@insp.cd

Tel: +243815800288

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